NCLEX-RN MASTERCLASS

NCLEX-RN Trending Topics Masterfile

Systems 1-12 · A Comprehensive Analysis of High-Yield NCLEX-RN Trends and Case Studies (2026)

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THE ROADMAP

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01

SYSTEM 01

Endocrine & Metabolic

Highest-Yield



Endocrine & Metabolic

01	Diabetes Mellitus — DKA vs HHNS — "Case: DM (DKA) and DM (T2). Not HHNS because sugar was below 200." (clarified) — Formats: Case Study (multiple), prioritization within case, 4-clients-in-1 NGN set including DKA/HHNS; standalone question tying DM2 → risk for HHNS.
02	Hyperthyroidism vs Hypothyroidism — "1st question about hyperthyroidism — most significant finding: palpitations, irritability." — Formats: Case Study; Bowtie: Hypothyroidism (parameters/monitoring); complication focus: Myxedema Coma.
03	Thyroidectomy — post-op risks and signs/symptoms (airway/bleeding/thyroid storm linkage). Formats: Stand-alone; SATA variations.
04	Puerperium vs Cytotec-Induced Fever — "Puerperal infection vs Cytotec-induced fever — case study." — Formats: Case Study (OB cross-over with endocrine prostaglandin effect).
05	Adrenal Disorders — Cushing vs Addison (compare/contrast), Adrenal Insufficiency → hyperkalemia, Pheochromocytoma, Adrenalectomy care. Formats: Case Study and Stand-alone.
06	Hyperemesis Gravidarum (electrolyte/ketosis link) – appears as OB case but repeatedly tied to hypokalemia/ketosis decisions. Formats: Case Study; SATA.

HIGH-FREQUENCY ITEMS (SATA / BOWTIE / STAND-ALONE)

Endocrine & Metabolic

01	DM Core — Insulin knowledge: peaks, Lispro + Glargine together? (compatibility/administration), insulin before meals/activity, hypoglycemia teaching for peds (9 y/o), carry glucagon.
02	DM Foot Care — SATA.
03	Dawn Phenomenon — Bowtie.
04	Type 1 vs Type 2 (treatment differences; crisis risk mapping).
05	Gluconeogenesis (basic concept recognition).
06	Thyroid storm (recognize most significant findings — tachycardia/palpitations, hyperthermia, neuro changes).
07	Hypothyroidism → Myxedema Coma (red-flag linkage).

Endocrine & Metabolic

01	DI vs SIADH — compare/contrast — Formats: SATA; Stand-alone.
02	Desmopressin (nasal) — nursing considerations (cross-tagged with Pharm).
03	Niacin (Vitamin B3) — flushing (SE teaching).
04	Rhabdomyolysis — what to monitor (CK, myoglobinuria, kidneys) — appears under metabolic/renal crossover.
05	Raloxifene (osteoporosis SERM — indications/risks).

PHARMACOLOGY ANCHORED TO ENDOCRINE

Endocrine & Metabolic

DRUG / CLASS	KEY NCLEX TEACHING
Insulins: Lispro, Glargine, NPH	Know peaks; IV regular for DKA; do not shake; rotate sites.
Lithium	Toxicity set (also flagged "hold HCTZ"; sodium/fluid counseling).
Desmopressin (DDAVP)	Nasal use, water intoxication risk (SIADH-like), I&O, sodium.
Raloxifene	Osteoporosis; DVT risk counseling.
ACE/ARB	Do not combine (electrolyte/renal implications).
Statins	Rhabdomyolysis watch-outs (dark urine, muscle pain).
Triptans (Sumatriptan/Eletriptan)	Migraine (vasoconstriction cautions in CAD).

Endocrine & Metabolic

01 Adrenal meds context: steroid teaching (hyperglycemia, infection risk); recognize Addisonian crisis triggers.

PICTURE / IMAGE-BASED PROMPTS

Endocrine & Metabolic

01	None uniquely endocrine-only in your dump, but skin/foot images for DM and Brudzinski/Kernig (neuro) repeatedly appeared. (Kept here as a cross-note for DM foot checks.)

PRIORITIZATION/DELEGATION HOOKS INSIDE ENDOCRINE ITEMS

Endocrine & Metabolic

01 DKA vs HHNS: who to assess first; fluids → insulin → potassium sequencing inside cases.

02 Thyroid storm/Myxedema coma: airway, perfusion, neuro status take priority.

03 Lithium toxicity: immediate hold + draw level + assess neuro/renal — urgent escalation.

04 Desmopressin water intoxication: intervene for hyponatremia signs (headache, confusion, seizures).

FREQUENTLY REPEATED TOPICS

Endocrine & Metabolic

"Pinaka-maraming lumabas: DM, OB, Alzheimer's — doon lang po umikot questions ko."

"Case: DM (DKA) and DM (T2) — hindi HHNS kasi sugar <200."

"Hypothyroidism — Bowtie (parameters, s/sx); complication: Myxedema Coma."

"Thyroidectomy s/sx after surgery."

"DI vs SIADH — comparison."

"DM foot care — SATA."

"Dawn phenomenon — Bowtie."

"Lithium toxicity — hold HCTZ; diet/fluids."

"Desmopressin (nasal) — nursing considerations."

"Raloxifene (osteoporosis) — DVT risk."

"Statins — when to take; watch for rhabdomyolysis."

"Triptans for migraine (e.g., Sumatriptan/Eletriptan)."

02

SYSTEM 02

Obstetrics & Newborn (OB/NB)



Obstetrics & Newborn (OB/NB)

01	Uterine Atony — "Case: uterine atony." — "Puerperal infection vs Cytotec-induced fever — case study."
02	Cytotec-Induced Fever (vs infection) — "Cytotec-induced fever (case); differentiate from puerperal infection."
03	PIH / Preeclampsia–Eclampsia / HELLP — "PIH (case study)." — "HELLP (case study)."
04	Labor & Delivery Composite Cases — "OB — pregnant 27 y/o: phases of labor." — "39 weeks, bleeding — what to follow up?" — "Shoulder dystocia; vacuum-assisted delivery (case)." — "PROM at 36 weeks — which medication is contraindicated (choices recalled: magnesium sulfate, oxytocin, terbutaline, penicillin G)."
05	Fetal Monitoring / Fetal Status — "Uterine rupture — effects on the fetus (SATA)." — "Variable deceleration — management (SATA)." — "Non-stress test; contraction test."
06	Neonatal Jaundice / Phototherapy / Kernicterus — "Phototherapy (case)." — "Kernicterus — baby position (case/image)."
07	Four-in-One NGN set (OB item inside) — "4 clients in one NGN — includes anemia/post-fall, dehydration infant, COPD adult, total hip — OB elements noted in other sets."

HIGH-FREQUENCY ITEMS — POSTPARTUM & HYPERTENSIVE DISORDERS

Obstetrics & Newborn (OB/NB)

01 Postpartum / Puerperium — "Puerperium (postpartum topics)." — "Lochia." — "Uterine inversion (recognition/management)."

02 Hypertensive Disorders of Pregnancy — "Pre-eclampsia / eclampsia — recognition and actions." — "PIH complication focus."

03 Obstetric Procedures & Monitoring — "Non-stress test (NST) and Contraction Stress Test (CST)." — "Rupture of membranes — fetal parts on palpation (ROM finding)."

04 OB Medications — "Cytotec-induced fever (differentiation)." — "Group B Streptococcus in labor — intrapartum antibiotic context appears in several recalls." — "TDAP (case mention)."

Obstetrics & Newborn (OB/NB)

01 Antepartum / Intrapartum Safety — "Preterm labor — true symptoms." — "Stages of labor." — "LION rule during bleeding (noted in a recall: 'Pregnancy (post-op) bleeding — LION')."

02 Maternal–Fetal Blood/Immunity — "Rh factor scenario: baby A+, mother O− — timing and follow-up (includes prior RhoGAM at 28 weeks; 'what's abnormal VS?')." — "Indirect Coombs test."

03 Newborn / Immediate Care — "Position for oral care in the unconscious client — side-lying (image-based; cross-listed but used in NB care)." — "Newborn phototherapy; kernicterus positioning." — "Car seat location appropriate for baby." — "Normal head size (NB)."

PICTURE / IMAGE-BASED PROMPTS

Obstetrics & Newborn (OB/NB)

01	"Kernicterus — baby position (picture)."
02	"Positioning image: oral care in unconscious client — side-lying."
03	"FHR/monitoring cues embedded in variable deceleration sets (worded, not necessarily pictured)."

OB-LINKED MEDS / PROCEDURES

Obstetrics & Newborn (OB/NB)

01 "Cytotec-induced fever."

02 "Penicillin G for GBS (intrapartum)."

03 "PROM at 36 weeks — contraindicated med among: magnesium sulfate, oxytocin, terbutaline, penicillin G (format recall)."

04 "TDAP — case mention."

05 "Rhogam timing noted ('at 28 weeks given')."

PRIORITIZATION / DELEGATION HOOKS

Obstetrics & Newborn (OB/NB)

01	"Uterine atony — immediate actions / who to see first."
02	"Third-trimester bleeding at 39 weeks — follow-up priority."
03	"Variable decelerations — sequence of interventions (SATA)."
04	"PROM with medication safety — what to hold / give first."
05	"GBS in labor — safety timing (antibiotic)."
06	"Postpartum hemorrhage vs Cytotec fever — differentiate and escalate correctly."

FREQUENTLY REPEATED TOPICS

Obstetrics & Newborn (OB/NB)

"Uterine atony — case study (multiple mentions)."

"PIH / Pre-eclampsia / Eclampsia — case studies; HELLP noted."

"Variable deceleration — management (SATA)."

"PROM at 36 weeks — which medication is contraindicated (choices included magnesium sulfate, oxytocin, terbutaline, penicillin G)."

"Rh factor case (baby A+, mother O–; Indirect Coombs; RhoGAM timing mentioned)."

"Stages of labor; Lochia; Puerperium focus."

"Cytotec-induced fever — differentiate from puerperal infection (case)."

"Shoulder dystocia; vacuum-assisted delivery (case)."

"Uterine rupture — fetal effects (SATA)."

"NST and CST (recognition and actions)."

"Phototherapy; Kernicterus — positioning (picture)."

03

SYSTEM 03

Reproductive & Maternal Health (GYN & Postpartum Disorders)



Reproductive & Maternal Health

01	Uterine Atony (Postpartum Hemorrhage) — "Case study: Uterine Atony – Cytotec-induced fever." — "Case study: Uterine Inversion and Atony." — Focus: boggy uterus, massage first, administer uterotonics (Oxytocin, Methergine, Cytotec). Format: NGN / Case Study / SATA.
02	Puerperal Infection vs Cytotec-Induced Fever — "Case: Puerperal infection vs Cytotec-induced fever differentiation." — Focus: differentiate drug-induced fever (no foul lochia, normal WBC) vs infection (foul smell, tenderness, ↑WBC). Format: Case Study / SATA.
03	Uterine Inversion — "Case: Uterine inversion (emergency postpartum)." — Focus: replace uterus manually, avoid fundal pressure. Format: Case Study / Prioritization.
04	Endometriosis — "Case: Endometriosis – chronic pelvic pain, dysmenorrhea." — Focus: infertility risk, management via hormonal therapy or laparoscopy. Format: NGN / Stand-alone.
05	Uterine Prolapse — "Case: Uterine prolapse – pelvic floor weakness." — Focus: Kegel exercises, avoid heavy lifting, possible pessary support. Format: Case Study / SATA.
06	PAD / PVD in GYN Setting — "Case: PAD/PVD questions 4–6 items (stand-alone and SATA)." — Often linked to menopausal women on HRT or raloxifene use. Format: NGN / SATA.

Reproductive & Maternal Health

TOPIC	KEY NCLEX FOCUS
Oral Contraceptives	Contraindicated in smokers >35 yrs, DVT risk; report calf pain or SOB.
IUD (Intrauterine Device)	Teach: monitor string monthly, report foul discharge or severe pain.
Barrier Methods	Prevent STI; diaphragm must be left in 6 hrs after intercourse.
Emergency Contraception	Take within 72 hrs; nausea common side effect.
Hormonal Therapy (HRT)	Raloxifene or estrogen therapy: DVT risk warning.
Sterilization (Tubal Ligation / Vasectomy)	Permanent; alternate contraception for 3 months until sperm count confirmed.

Reproductive & Maternal Health

DISORDER	NCLEX-LINKED POINTS
Endometriosis	Ectopic endometrial tissue; pelvic pain; infertility risk; laparoscopy diagnostic.
Polycystic Ovarian Syndrome (PCOS)	Irregular menses, hirsutism, infertility; manage with metformin and weight control.
Premenstrual Syndrome (PMS)	Mood swings, bloating; non-pharmacologic relief emphasized.
Menopause	Hot flashes, night sweats; teach non-hormonal relief, monitor bone density.

C. POSTPARTUM / REPRODUCTIVE COMPLICATIONS

Reproductive & Maternal Health

01	"Uterine rupture – SATA (effects on fetus)."
02	"Shoulder dystocia – vacuum-assisted delivery case (cross with OB)."
03	"Hyperemesis gravidarum – hypokalemia SATA."
04	"Preterm labor – medication contraindicated ($MgSO_4$, Oxytocin, Terbutaline, Penicillin G)."
05	"PIH / HELLP – case study crossover."
06	"Anemia in pregnancy – SATA."
07	Format: SATA / Prioritization / NGN Case.

Reproductive & Maternal Health

CONDITION	KEY NCLEX FOCUS
Breast Cancer	SATA – risk factors: early menarche, nulliparity, family history, late menopause.
Cervical Cancer	Linked to HPV; preventive teaching on Pap smear schedule.
Ovarian Cancer	Silent symptoms (bloating, pelvic pain).
Teletherapy (Cervical CA)	Safety precautions – private room, limit exposure time, shielding.
Oral Cancer	"Painless white patch (leukoplakia)."

E. REPRODUCTIVE PHARMACOLOGY

Reproductive & Maternal Health

DRUG	PURPOSE / KEY NCLEX TEACHING
Cytotec (Misoprostol)	Uterotonic for atony; prostaglandin effect may cause fever (differential diagnosis item).
Methergine (Methylergometrine)	Increases uterine tone; contraindicated in hypertension.
Oxytocin (Pitocin)	Induces labor; monitor for hyperstimulation, fetal distress.
Raloxifene	Osteoporosis prevention; DVT risk counseling.
Tamoxifen	Breast CA; hot flashes, endometrial CA risk.
Magnesium Sulfate	For PIH seizure prophylaxis; monitor DTR and RR.

PICTURE / IMAGE-BASED PROMPTS

Reproductive & Maternal Health

01	Uterine inversion diagram (emergency recognition).
02	Breast self-exam visual (hand position during inspection).
03	Fetal monitor strips showing variable deceleration (cross OB topic).
04	Stage of lochia photos (rubra → serosa → alba).

PRIORITIZATION & DELEGATION HOOKS

Reproductive & Maternal Health

01 "Which postpartum client to see first?" → Boggy uterus or excessive bleeding.

02 "Which medication is contraindicated?" → $MgSO_4$ + Oxytocin together.

03 "Differentiation question – Cytotec fever vs Puerperal infection."

04 "Delegation – LPN to monitor stable postpartum; RN handles hemorrhage."

05 "Variable decelerations – intervene first (reposition, O_2, stop oxytocin)."

FREQUENTLY REPEATED TOPICS

Reproductive & Maternal Health

"Case: Uterine atony – Cytotec-induced fever."

"Case: Puerperal infection vs Cytotec fever."

"Case: Uterine prolapse – Kegel exercises."

"Cytotec-induced fever – differentiate from infection."

"Oxytocin – monitor for fetal distress."

"Uterine rupture – SATA fetal effects."

"Case: Uterine inversion."

"Case: Endometriosis – chronic pelvic pain."

"Breast cancer risk factors – SATA."

"Methergine – contraindicated in hypertension."

"Teletherapy (Cervical CA) – radiation safety."

"Preterm labor meds – contraindications ($MgSO_4$, Terbutaline)."

04

SYSTEM 04

Cardiovascular

Very High Yield

Cardiovascular

01	Congestive Heart Failure (CHF) — "Case: CHF – digoxin (hypokalemia); 3 questions similar." — "CHF case appeared with other comorbidities in 4-client set (hip replacement, COPD, anemia)." — Formats: Case Study / NGN; prioritization within case.
02	Myocardial Infarction (MI) — "Case: MI vs sinus tachycardia interpretation (EKG context)." — Formats: Case Study; image/exhibit (EKG tracing recognition).
03	Cardiac Compartment Syndrome (NGN 4-case set) — "33 y/o male with compartment syndrome (case with anemia and post-fall)." — (Musculoskeletal overlap but cardio-perfusion implication noted.)
04	Pericardial Tamponade / Post-Cardiac Surgery — "Cardiac tamponade (case within priority sets)."

HIGH-FREQUENCY ITEMS — HEART FAILURE & CARDIAC FUNCTION

Cardiovascular

01	Left-Sided Heart Failure Findings — SATA — Dyspnea, crackles, orthopnea, fatigue.
02	CHF – Digoxin focus — Check K^+ before giving; toxicity when hypokalemia present.
03	Recognize early toxicity: anorexia, nausea, visual changes (yellow-green halos).
04	Furosemide vs Albuterol — "Coarse crackles and SOB — differentiate loop diuretic vs bronchodilator."
05	Spironolactone (best time to administer) — Morning to avoid nocturia.

Cardiovascular

01	Atrial Flutter (EKG strip — interpretation).
02	Atrial Fibrillation (EKG tracing — management).
03	Ventricular Tachycardia and V-Fib (EKG strips). — "V-Fib meaning — cardiac output ceased." — "Management of A-Fib and V-Tach — 2 EKG questions."
04	PVCs — warning sign of electrolyte imbalance.
05	PEA (ECG photo — asystole-like strip).
06	ST Elevation, Atrial Fib, 1st Degree HB, PVC — Prioritization set.

Cardiovascular

01	CAD / Angina / MI Management. — "MI case; recognize sinus tach vs ischemic pattern." — "Angina pectoris – teaching item."
02	Peripheral Artery / Venous Diseases (PAD / PVD). — "PAD management — exercise, dependent positioning, no crossing legs." — "PVD – DVT risk counseling."
03	Deep Vein Thrombosis — SATA / Case Study. — Signs and symptoms: warmth, pain, swelling, Homan's sign (old reference trap).
04	Pulmonary Embolism — Signs and Symptoms. — Dyspnea, tachypnea, pleuritic pain, tachycardia.

Cardiovascular

01 Distributive Shock — Bowtie.

02 Cardiac Tamponade — Beck's Triad (low BP, muffled sounds, JVD).

03 Neurogenic Shock vs Autonomic Dysreflexia (overlap in neuro set).

04 Impending doom sensation — priority intervention (PE / MI context).

KEY PHARMACOLOGY UNDER CARDIO

Cardiovascular

DRUG / CLASS	KEY NCLEX TEACHING
Digoxin	Hold if HR < 60; monitor K^+.
ACE Inhibitors & ARBs	Do not combine; risk of hyperkalemia.
Beta-Blockers (e.g., Acebutolol)	Teach to check pulse before taking.
Niacin (Vit B3)	Flushing side effect.
Statins (Simvastatin / Atorvastatin)	Watch for rhabdomyolysis.
DOACs (e.g., Apixaban)	Indicated vs not indicated points noted.
Enoxaparin	No aspiration; do not remove air bubble; inject SubQ.

Cardiovascular

01	Vasodilators – monitor for orthostatic hypotension.
02	Diuretics (Loop and K-Sparing) – fluid status and electrolytes.
03	EKG strips – A-Fib, A-Flutter, V-Fib, V-Tach. (Picture / Image-Based Prompts)
04	Heart failure finding sets (Left vs Right SATA worded, not imaged). (Picture / Image-Based Prompts)
05	"Who to assess first" within CHF/MI clients (airway-breathing-circulation). (Prioritization / Delegation Patterns)
06	Delegation of UAP to perform ADLs for stable cardiac clients. (Prioritization / Delegation Patterns)
07	LPN scope question — "Which task can be delegated (suctioning)." (Prioritization / Delegation Patterns)
08	Cardiac clients within multi-scenario sets often used for triage hierarchy. (Prioritization / Delegation Patterns)

FREQUENTLY REPEATED TOPICS

Cardiovascular

"CHF and digoxin (hypokalemia) — 3 similar items."

"Left-sided HF findings — SATA."

"V-Fib photo — cardiac output ceased."

"Pulmonary embolism signs and symptoms."

"DVT signs and symptoms — SATA."

"ACE + ARB – do not combine."

"Statins – rhabdomyolysis warning."

"DOACs (Apixaban) – indication vs contraindication."

"MI vs sinus tachycardia (EKG interpretation case)."

"A-Fib and A-Flutter EKG strips."

"Cardiac tamponade recognition."

"PAD / PVD management."

"Spironolactone (best time to administer)."

"Niacin flushing."

"Enoxaparin – no aspiration / keep air bubble."

05

SYSTEM 05

Neurologic & Psychiatric Nursing

Very High Yield

Neurologic & Psychiatric Nursing

01	Alzheimer's Disease — "Case study: Alzheimer's disease." — Frequently paired with therapeutic communication, caregiver exhaustion, and safety precautions (wandering, sundowning). Format: Case Study / NGN, SATA for safety & orientation cues.
02	Schizophrenia — "Case study: Schizophrenia (2 cases)." — "Positive and negative symptoms – SATA." — "Schizo case: contractual relationship, voluntary admission 72-hour rule." Format: Case Study / SATA / Stand-alone.
03	Anxiety Disorders — "Case study: anxiety disorder." — "Case study: Asthma vs Anaphylactic shock triggered by vaccine—psych element of anxiety overlay." Format: Case Study (psych + med integration).
04	Psych – Risperidone — "Case study: Psych (Risperidone)." — Focus: EPS, patient safety, med adherence. Format: Case Study.

Neurologic & Psychiatric Nursing

01	Neurodegenerative Disorders — "Multiple Sclerosis (MS), Myasthenia Gravis (MG), Parkinson's Disease, Trigeminal Neuralgia — who is most at risk for anxiety?" — "Case: Parkinson's Disease (NGN)." — "Degenerative disorders: MS, GBS, ALS — SATA." Format: Case Study / SATA / Stand-alone.
02	Neuro Case Cluster — "Case: Spinal injury (cervical)." — "Case: Increased ICP / Cushing triad." — "Case: Kernicterus (baby positioning)." Format: Case Study / Picture-based / SATA.
03	NMS / Serotonin Syndrome / EPS — "Case: DIC, Kernicterus, NMS, Peritonitis (multi-case NGN)." Format: Multi-scenario NGN.

Neurologic & Psychiatric Nursing

01	"Risperidone (EPS risk), Cogentin (anticholinergic)."
02	"Cogentin used to counter EPS."
03	"Haldol, Olanzapine — adverse effects."
04	"NMS recognition – rigidity, fever, diaphoresis, altered mental status."
05	"Fluphenazine — post-herpetic neuralgia confusion in some reports (verify cross)."
06	"Lithium — toxicity and diet."
07	"Lithium – hold HCTZ; monitor sodium/fluid."
08	"Lithium toxicity – confusion, coarse tremor, vomiting."

Neurologic & Psychiatric Nursing

01	"SSRIs (Fluoxetine) — for OCD."
02	"MAOIs – avoid tyramine."
03	"TCAs (Elavil) — sedation, anticholinergic effects."
04	"Bupropion — used in GAD; avoid with seizure history."
05	"Niacin confusion corrected: Vitamin B3 flushing – not psych, but same exam domain."
06	"Lorazepam — priority: assess sedation, avoid abrupt stop."
07	"Flumazenil — antidote for benzodiazepine overdose."
08	"ECT (Depakene)." — "ECT – consent, NPO, remove dentures, post-ECT orientation." — "Depakene – hold before ECT (seizure threshold)."

NEUROLOGIC DISORDERS (NON-PSYCH)

Neurologic & Psychiatric Nursing

01 Stroke / CVA – "Right-sided stroke — care plan: support affected side, assess dysphagia, gait belt use."

02 Increased ICP / Cushing Triad – bradycardia, irregular respirations, widened pulse pressure.

03 Autonomic Dysreflexia – "Full bladder trigger."

04 Neurogenic Shock – "Bradycardia, hypotension, tachypnea."

05 Meningitis – "Brudzinski & Kernig sign — picture recognition." — "Cohorting rules — meningococcal vs viral."

06 Delirium Tremens – "Alcohol withdrawal case."

07 Delirium vs Alzheimer's — classification question.

08 *Glasgow Coma Scale — interpretation.

09 Epilepsy / Seizure Precautions — which is NOT included? (SATA).*

NEW ADDITION — ALICE IN WONDERLAND SYNDROME (AIWS)

Neurologic & Psychiatric Nursing

FEATURE	CLINICAL DETAIL
What it is	A neurological perceptual disorder (also called Todd's Syndrome) causing distorted visual perception of body size, shape, or surrounding objects — NOT a psychiatric or ophthalmologic condition.
Hallmark presentations	Micropsia (objects/body look too small), macropsia (too large), metamorphopsia (shapes distorted), and altered sense of time or spatial distance.
Most common trigger	Migraine — especially in children and adolescents; AIWS episodes often occur just before, during, or after a migraine (an aura-like phenomenon).
Other associated causes	Epstein-Barr virus / infectious mononucleosis, temporal lobe epilepsy, sleep deprivation, and rarely brain lesions or tumors (red flag if new-onset in an adult without migraine history).
Key differentiator from psychosis	Insight is preserved — the client KNOWS the perception is distorted and not real. This is the opposite of schizophrenia, where reality testing is impaired.

AIWS — WHY IT'S TRENDING & KEY POINTS TO REMEMBER

Neurologic & Psychiatric Nursing

01	Why it's trending on NCLEX: The 2026 NCLEX leans heavily into Clinical Judgment — AIWS is a perfect "safe vs. unsafe thinking" trap item because a nurse who doesn't recognize it may mistake it for a psychotic break and request antipsychotic medication, which would be the UNSAFE answer.
02	Priority nursing action #1 — Reassure & orient. Since insight is intact, calmly validate that what they're experiencing is a real neurological symptom, not "going crazy"; this reduces fear and supports accurate self-report.
03	Priority nursing action #2 — Assess for the underlying cause. Ask about recent migraine history, headache pattern, recent viral illness (EBV/mono symptoms — fever, fatigue, sore throat, lymphadenopathy), sleep history, and seizure activity.
04	Priority nursing action #3 — Safety first. Distorted body/spatial perception is a fall and injury risk — treat it like any other perceptual/neuro symptom: clear the environment, assist with ambulation, and do not leave the client unattended during an active episode.
05	Priority nursing action #4 — Rule out red flags. New-onset AIWS in an adult with NO migraine history, or accompanied by focal neuro deficits, warrants prompt neuro workup to rule out a structural lesion or tumor — do not simply reassure and discharge.
06	Common NCLEX trap to avoid: selecting an antipsychotic (e.g., Risperidone, Haldol) or a psychiatric hold as the answer — AIWS is neurological, and the correct action targets the trigger (migraine management, supportive care for viral illness), not psychiatric treatment.
07	Memory hook: "AIWS = Alice's world is small, but her insight stays whole." Distorted perception + intact insight + migraine/EBV link = AIWS, not psychosis.

THERAPEUTIC COMMUNICATION (CROSS-TOPIC TREND)

Neurologic & Psychiatric Nursing

01	"Thera comm – frequently repeated."
02	"Conflict with staff (how to manage)."
03	"Patient assignment for floaters (MS to Psych)."
04	"Conflict management, reinforcement, slander."
05	Formats: Stand-alone, SATA, prioritization (psych + management).

OTHER FREQUENT MENTAL HEALTH TOPICS

Neurologic & Psychiatric Nursing

01	Personality Disorders — "Histrionic personality disorder — stand-alone." — "Narcissistic personality disorder — situational item." — "Borderline personality disorder — SATA / scenario."
02	Depression / Suicide — "Depression management; ECT indication." — "Suicide precautions (frequent mention)."
03	Eating Disorders — "Bulimia nervosa — Russell sign (SATA)." — "Anorexia nervosa — body image and safety link."
04	PTSD — "PTSD symptoms — SATA."
05	Defense Mechanisms — "Rationalization — identify in scenario."
06	Substance Use / Withdrawal — "Alcoholism – delirium tremens." — "Opioid intoxication – pinpoint pupils, respiratory depression." — "Heroin intoxication – NCLEX classic; confirmatory." — "Methadone – substitution therapy (cross-mentioned)."
07	Cognitive / Behavioral — "ADHD — loss of appetite, weight loss." — "Autism — repetitive behavior, poor eye contact."

PICTURE / EXHIBIT PROMPTS

Neurologic & Psychiatric Nursing

01	Kernig sign / Brudzinski sign – picture identification.
02	Global aphasia – scenario image.
03	Meningitis / phototherapy babies – visual cues.
04	EKG exhibits (neuro-cardiac overlap).

PRIORITIZATION / DELEGATION THEMES

Neurologic & Psychiatric Nursing

01 "UAP delegation – SATA (5 rights of delegation)."

02 "Psych crisis: who to assess first (suicidal vs hallucinating client)."

03 "Client with visual hallucinations – appropriate response."

04 "Which call to return first – prioritization."

05 "Hospice vs palliative (case & SATA)."

FREQUENTLY REPEATED TOPICS

Neurologic & Psychiatric Nursing

- *"Case: Alzheimer's disease (NGN)."*
- *"Positive and negative symptoms – SATA."*
- *"Case: Psych – Risperidone."*
- *"ECT – safety, pre/post procedure."*
- *"Depression, bipolar, borderline – management."*
- *"Histrionic and narcissistic personality disorders."*
- *"Alcoholism – delirium tremens; opioid intoxication."*
- *"CVA care plan – dysphagia and safety."*
- *"Increased ICP – Cushing's triad SATA."*
- *"Seizure precautions – SATA."*
- *(NEW ADDITION) "AIWS (Alice in Wonderland Syndrome) — distorted body/visual perception with intact insight; linked to migraine and EBV; do not mistake for psychosis."*
- *"Case: Schizophrenia (2 sets)."*
- *"Case: Anxiety disorder."*
- *"Therapeutic communication and conflict management."*
- *"NMS – rigidity, fever, diaphoresis, altered mental status."*
- *"Bulimia – Russell sign."*
- *"Defense mechanism – rationalization."*
- *"Delirium vs Alzheimer's classification."*
- *"Autonomic dysreflexia – full bladder trigger."*
- *"Brudzinski/Kernig – picture recognition."*
- *"UAP delegation – 5 rights SATA."*

06

SYSTEM 06

Safety, Infection Control, Prioritization, Delegation, and Management of Care



Safety, Infection Control, Prioritization, Delegation, and Management of Care

01	Disaster Management / Triage — "Disaster management – who to save first." — "Imminent tornado at the hospital – move patients away from windows." — "Disaster preparedness (Family Disaster Plan)." — "Discharge readiness during disaster." — "Triage in ER and outside – prioritization cases." Format: Case Study / NGN with drag-and-drop or ranking sequence (who to save first).
02	Four-Client Scenario (Priority NGN) — "4 clients in one NGN (Pneumonia, MI, COPD, Anemia/Post-Fall)." — "Prioritize who to see first/last." — "Another 4-case set with CHF, COPD, dehydration infant, hip replacement – prioritization theme."
03	Hospice and Palliative Care — "Case study – Hospice care vs Palliative care." — "COPD end-of-life scenario." Format: NGN multi-scenario or ethical decision item.

A. INFECTION CONTROL / ISOLATION PRECAUTIONS

Safety, Infection Control, Prioritization, Delegation, and Management of Care

01	C. diff – contact precaution; soap & water hand hygiene only.
02	MRSA / VRSA – contact precautions.
03	RSV – contact and droplet (mask + gown + gloves).
04	Tuberculosis – airborne (N95, negative pressure).
05	Meningitis – droplet; cohort with same infection only.
06	Varicella / Chickenpox / Herpes Zoster – airborne + contact until lesions crusted.
07	Salmonellosis / Shigellosis / Cholera – contact precautions.
08	Scarlet Fever / Fifth Disease / Rubeola – droplet precautions.

Safety, Infection Control, Prioritization, Delegation, and Management of Care

01	Pertussis – droplet; antibiotic prophylaxis for contacts.
02	HIV & Hepatitis B/D – standard precautions (no cohorting with infectious airborne cases).
03	Lyme Disease – contact precaution (picture question).
04	Hand Hygiene – before & after client contact; alcohol-based rub except with C. diff.
05	Formats: SATA / Matching / Picture-based (precaution sign images).

B. PRIORITIZATION PRINCIPLES

Safety, Infection Control, Prioritization, Delegation, and Management of Care

01	Keywords to watch: "First," "Immediate," "Intervene," "Priority," "Most concerning."
02	Common sets: "Telephone call – who to call back first." — "Which client to assess first." — "Who to discharge first." — "Which post-op patient to monitor closely." — "Which fall to report first."
03	Used repeatedly in NGN 4-case clusters with COPD, DKA, hip arthroplasty, anemia, etc.
04	Framework recall: Airway → Breathing → Circulation → Safety → Pain → Education sequence applied consistently.
05	Maslow + ABC + Safety hierarchy embedded in NGN scoring.

C. DELEGATION & ASSIGNMENT

Safety, Infection Control, Prioritization, Delegation, and Management of Care

01 Delegation SATA – 5 Rights of Delegation — Right Task, Circumstance, Person, Direction/Communication, Supervision/Evaluation.

02 Examples recalled: "Delegation of UAP – SATA." — "Delegation to LPN – suctioning." — "Assigning float nurse from Maternity to Medical-Surgical – which patient to assign?" — "Patient assignment for newly hired nurse – 2 weeks OB experience (which labor case?)."

03 UAP allowed: Vital signs, hygiene, ambulation, I&O, ADLs, stable clients.

04 LPN allowed: Reinforcement teaching, suctioning, dressing changes, stable post-ops.

05 Format: SATA / Drag-and-drop assignment scenarios.

D. SAFETY & RISK REDUCTION

Safety, Infection Control, Prioritization, Delegation, and Management of Care

01	Fall precautions, bleeding precautions, neutropenic precautions – SATA.
02	Positioning questions (oral care in unconscious client → side-lying; after procedures).
03	Fire / Disaster drills – move clients horizontally, away from danger.
04	Transferring client from bed to chair – place chair on strong side.
05	Blood transfusion reaction – immediate action: stop infusion.
06	Infiltration – secure IV with padded board; remove line if extravasation suspected.
07	Restraints – check every hour, tie to bed frame not side rails, quick-release knot.
08	Incident Reporting – when to file, objective documentation only.
09	Ethics overlap: breach of confidentiality, slander, client rights to refuse.

E. COHORTING / ROOM ASSIGNMENT & F. DISASTER PREPAREDNESS

Safety, Infection Control, Prioritization, Delegation, and Management of Care

01 "Cohort or room assign for gastro & C. diff; Hep B & D; HIV & Cancer; TB & Meningitis."

02 Principles recalled: Only cohort clients with same organism. Immunocompromised clients (e.g., chemo, HIV, transplant) should not share with infectious clients. New postop or sterile procedures → private room.

03 "Family Disaster Plan teaching." — "Ethical duty of nurses during disasters (serve nearby areas)." — "Ask older adults to prepare 30-day supply." — "Triage: who to see first, who can wait." — "LION rule during bleeding emergencies in pregnancy." — "ER triage for multiple trauma cases – prio and dele combined."

Safety, Infection Control, Prioritization, Delegation, and Management of Care

01	"Conflict with staff – how to manage."
02	"Reinforce vs Remind — differentiating supervision levels."
03	"Caregiver exhaustion – refer to support group."
04	"Patient complaints / incident reporting."
05	"Slander / breach of confidentiality."
06	Format: Scenario / Select-all / Ethical decision.

FREQUENTLY REPEATED TOPICS

Safety, Infection Control, Prioritization, Delegation, and Management of Care

"Disaster management – who to save first."

"Triage (ER and outside) – prioritization."

"Infection control – C. diff, MRSA, RSV, TB, Meningitis."

"Cohorting principles (infectious vs immunocompromised)."

"Five rights of delegation – SATA."

"Delegation to UAP / LPN – ADLs, suctioning."

"Fall precautions / bleeding precautions / neutropenic precautions – SATA."

"Blood transfusion reaction – stop infusion."

"Infiltration – secure with padded board."

"Incident report – objective documentation."

"Fire / tornado / earthquake drills – safety moves."

"Ethical violations – confidentiality, slander."

"Conflict management / therapeutic communication."

"Hospice vs palliative care – case."

07

SYSTEM 07

Respiratory Nursing

Very High Yield & Case-Heavy



Respiratory Nursing

01	Asthma (viral-triggered vs anaphylactic shock) — "Case study: Asthma triggered by viral infection vs anaphylactic shock after vaccine." — Focus: Airway assessment, epinephrine administration, early recognition of respiratory distress. Format: NGN / Multi-scenario case; prioritization within case.
02	COPD (Chronic Obstructive Pulmonary Disease) — "Case study: COPD with pneumonia." — "4-client NGN set includes 67 y/o male COPD on O_2 at 3 LPM, requesting nebulizer — who to see first." — "Hospice care with COPD – case." Format: NGN / Prioritization / Ethical (end-of-life).
03	Pneumonia (Adult / Pediatric) — "Pneumonia case study – isolation precautions, airway clearance." — "Drag-and-drop: Pneumonia vs PTB (Tuberculosis) differentiation." Format: NGN / Matching / SATA.
04	Pulmonary Embolism (PE) — "Pulmonary embolism signs and symptoms – SATA." — "Impending doom – priority intervention." Format: Case / SATA / Stand-alone.
05	Respiratory Failure / Ventilator Management — "Mechanical Ventilator – high pressure alarm question." — "ABG interpretation – stand-alone item." — "Bowtie: Distributive Shock / Respiratory support decision." Format: Bowtie / Case Study / Prioritization.
06	Infectious Respiratory Diseases — "C. diff, MRSA, RSV (precautions)." — "Transmission-based precautions – TB." — "Cohorting: TB & Meningitis not together." — "Group B Strep infection in pregnant client (newborn respiratory risk)." Format: SATA / Picture / Scenario.

Respiratory Nursing

01 Asthma: rescue vs maintenance meds; triggers (viral/allergen); peak flow use.

02 COPD: pursed-lip breathing, avoid high O_2, airway clearance.

03 Pneumonia: productive cough, crackles, fever, oxygenation priority.

04 Bronchial Asthma vs Anaphylaxis: identify shock features (hypotension, hives, wheezing).

Respiratory Nursing

01	"ABG interpretation (identify acid-base imbalance)."
02	"Distributive shock – Bowtie; ABG application."
03	"High-pressure ventilator alarm – check for obstruction, secretions, biting tube."
04	"Low-pressure alarm – disconnection or leak."
05	"Arteriogram – increase fluid intake 3L after procedure."
06	"Clean-catch urine (teaching) – avoid contamination."
07	"PICC line – cold extremity = complication."
08	(Note: these appear under Safety but frequently tied to respiratory/circulatory monitoring.)

Respiratory Nursing

01 "RSV – droplet/contact precaution."

02 "Varicella / Scarlet Fever / Fifth Disease – droplet or airborne."

03 "Viral encephalitis – neuro-respiratory monitoring."

04 "Pneumonia isolation – droplet."

05 "TB – airborne, N95, negative pressure room."

SIGNS & SYMPTOMS TO RECOGNIZE

Respiratory Nursing

01 Crackles: Inspiratory crackles SATA → Left-sided heart failure, pneumonia, COPD.

02 Aphasia (Global) & Hypoxia confusion: early sign of respiratory compromise.

03 Cyanosis, Restlessness, Tachypnea: early hypoxemia indicators.

RESPIRATORY PHARMACOLOGY

Respiratory Nursing

01	Bronchodilators: Salbutamol / Albuterol – rescue med, assess for tremors, tachycardia.
02	Ipratropium – anticholinergic; contraindicated with peanut allergy.
03	Anti-inflammatory: Prednisone / Corticosteroids – infection risk, taper off.
04	Mucolytics & Expectorants: Guaifenesin, Acetylcysteine (for paracetamol overdose also seen).
05	Antibiotics: Azithromycin / Erythromycin for Pertussis, Pneumonia.
06	Antihistamines / Antitussives: used in mild respiratory allergy sets.

PICTURE / IMAGE-BASED PROMPTS

Respiratory Nursing

01 EKG / oxygen saturation monitors – recognition of abnormal waveform.

02 Ventilator alarm exhibit (photo-based scenario).

03 Pursed-lip breathing visual.

04 PE chest x-ray–type question (rare mention).

PRIORITIZATION & DELEGATION HOOKS

Respiratory Nursing

01	"Which respiratory client to see first?" (COPD exacerbation vs stable pneumonia).
02	"Who can the UAP assist?" (stable COPD, no O_2 change).
03	"Which post-op client to monitor for PE?" (hip/knee arthroplasty).
04	"When to report coarse crackles vs diminished sounds."
05	"Airway obstruction sequence (head tilt–chin lift, suction, O_2)."

FREQUENTLY REPEATED TOPICS

Respiratory Nursing

"Case: Asthma – viral vs anaphylactic shock."

"Case: COPD with pneumonia – airway clearance priority."

"Case: Pulmonary embolism – SATA."

"Mechanical ventilator – high pressure alarm."

"ABG interpretation."

"RSV / TB / Varicella – isolation precautions."

"Inspiratory crackles – SATA."

"Pursed-lip breathing (COPD)."

"Arteriogram – increase fluid intake 3L."

"PICC line – cold extremity = complication."

"PE – impending doom feeling (priority)."

"Vent alarm – obstruction vs disconnection."

08

SYSTEM 08

Renal, Fluid & Electrolyte Management

High Frequency



Renal, Fluid & Electrolyte Management

01	Acute Kidney Injury (AKI) / Chronic Kidney Disease (CKD) — "Case study: AKI and CKD management – fluid restrictions and lab values." — "Oliguric phase of kidney injury – what to report." — "CKD – teaching diet, monitoring BUN/creatinine, avoid nephrotoxic drugs." Format: NGN / Case / Prioritization.
02	Rhabdomyolysis (cross renal-metabolic) — "Client at risk for rhabdomyolysis – what to monitor?" → CPK, myoglobinuria, urine output. Format: Case Study / SATA.
03	Adrenal Insufficiency (Hyperkalemia) — "Case – Adrenal insufficiency with hyperkalemia." — Focus on electrolyte monitoring and fluid balance. Format: Case Study / SATA.
04	DKA / HHNS Overlap (Cross Endocrine and Renal) — Seen in multi-system cases with fluid deficit, electrolyte shift, ABG analysis. Format: NGN / Bowtie.

Renal, Fluid & Electrolyte Management

01 Cystometrogram – purpose: assess bladder capacity and tone.

02 Left Hemiparesis client feeding position – feed on right side (to avoid aspiration).

03 Urine Specimen Collection – clean-catch teaching emphasized in several reports.

04 Paracentesis (Ascites) – measure abdominal girth and weight pre-/post-procedure.

05 Renal diet: low protein (acute phase), restricted Na^+, K^+, phosphate; adequate calories.

Renal, Fluid & Electrolyte Management

ELECTROLYTE	KEY SATA FINDINGS / NOTES
Hyperkalemia	Peaked T waves, weakness, GI hyperactivity; treat with calcium gluconate, insulin + D50, Kayexalate.
Hypokalemia	Seen with loop diuretics; ↑ digoxin toxicity risk.
Hypomagnesemia	SATA – tremors, hyperreflexia, positive Trousseau / Chvostek.
Hypermagnesemia	Depressed DTRs, bradycardia (if $MgSO_4$ toxicity in OB).
Hyponatremia / Hypernatremia	Mental status changes – priority assessment.
Calcium Imbalance	Trousseau's sign photo item – "what to do next" → initiate cardiac monitoring if option to assess Ca^{2+} not listed.

Renal, Fluid & Electrolyte Management

01	"Nursing diagnosis – risk vs actual fluid volume deficit."
02	"CHF client – daily weights to monitor fluid status."
03	"Oliguric phase AKI – report < 30 mL/hr output."
04	"Ascites management – measure, assess for respiratory distress."
05	ABG Interpretation – appears in Respiratory set but cross-tested with DKA/renal cases.
06	DKA pattern: Metabolic acidosis ↓pH, ↓HCO_3^-.
07	AKI compensation: Respiratory alkalosis or metabolic acidosis recognition.

E. RENAL-RELATED PHARMACOLOGY

Renal, Fluid & Electrolyte Management

DRUG	KEY NCLEX TEACHING
Furosemide	Loop diuretic; monitor K^+ loss, ototoxicity.
Spironolactone	K-sparing; avoid K supplements.
ACE / ARB	Nephroprotective but monitor K^+ and creatinine.
Calcium Gluconate	For severe hyperkalemia.
$MgSO_4$	Monitor DTR hourly; hold if absent.
Kayexalate	GI K^+ removal; monitor bowel sounds.
Desmopressin	For DI (renal water balance); risk of water intoxication.

F. PROCEDURAL SAFETY / PRECAUTIONS & CROSS-TOPIC CONNECTIONS

Renal, Fluid & Electrolyte Management

01	Post-Arteriogram – encourage 3 L fluid intake to flush contrast.
02	Dialysis (client care): check thrill/bruit, no BP/venipuncture on access arm, monitor hypotension.
03	Peritoneal Dialysis (stand-alone & SATA): infection signs, outflow clarity, peritonitis → cloudy effluent, abdominal pain.
04	Peritonitis (NGN Case) – recognition and management steps.
05	Endocrine link: DKA and HHNS → fluid resuscitation and electrolyte replacements.
06	Cardio link: CHF fluid overload vs renal failure retention.
07	OB link: $MgSO_4$ toxicity in pre-eclampsia → monitor DTR and respiration.

PRIORITIZATION & DELEGATION PATTERNS

Renal, Fluid & Electrolyte Management

01 "Which client to see first – AKI with K^+ 6.2 or CHF with crackles?"

02 "Who can be assigned to LPN – stable dialysis patient or post-HD hypotensive client?"

03 "Which lab value to report first – creatinine increase, BUN elevated, K^+ 6.0?"

04 "Post-dialysis – monitor for headache, confusion (sign of disequilibrium syndrome)."

FREQUENTLY REPEATED TOPICS

Renal, Fluid & Electrolyte Management

"Case: AKI / CKD management."

"Oliguric phase – what to report."

"Rhabdomyolysis – monitor CPK and urine output."

"Adrenal insufficiency – hyperkalemia."

"Hypomagnesemia – SATA findings."

"$MgSO_4$ toxicity – check DTR."

"Cystometrogram – bladder assessment."

"Peritoneal dialysis – cloudy effluent = infection."

"Arteriogram – increase fluids 3 L."

"Kayexalate – monitor bowel sounds."

"Calcium gluconate – antidote for hyperkalemia."

"Fluid volume deficit – actual vs risk NDx."

"Report $K^+ > 6.0$ immediately."

09

SYSTEM 09

Gastrointestinal & Hepatic Disorders

High Yield

Gastrointestinal & Hepatic Disorders

01	Peritonitis — "Peritonitis – NGN case." — "Peritonitis in 4-in-1 case; cloudy effluent in dialysis." — Focus: infection signs, abdominal distention, rebound tenderness, fever. Format: NGN / Case Study / SATA.
02	Acute Pancreatitis — "Case: Acute pancreatitis – Cullen's and Grey Turner's signs." — "Cullen's and Grey Turner's – SATA." Format: Case / SATA.
03	Cholecystitis / Cholelithiasis — "Case: Cholecystitis." — Focus: RUQ pain, fatty food intolerance, Murphy's sign, low-fat diet. Format: Case / Stand-alone.
04	Peptic Ulcer Disease — "Case: Peptic ulcer – NGN." — "Peptic ulcer (case in 4-set with DKA, phototherapy, anaphylaxis)." Format: NGN / Case Study.
05	Cirrhosis / Liver Failure — "Cirrhosis – flapping hand tremor (asterixis)." — "Liver cirrhosis with ascites – SATA." — "Hepatic encephalopathy – Rifaximin treatment." Format: Case / SATA / Drug recognition.
06	Diverticulitis — "Case: Diverticulitis – what to avoid." — "Diverticulitis – dietary fiber modification." Format: Case Study / SATA.
07	Gastrointestinal Bleeding — "Pregnancy with post-op bleeding – LION (Left side, IV, O_2, Notify)." — "GI bleed cases integrated with safety/prioritization." Format: Case / Prioritization.

A. DISORDERS AND KEY CLINICAL SIGNS

Gastrointestinal & Hepatic Disorders

DISORDER	KEY NCLEX-FOCUSED POINTS
Pancreatitis	Cullen's sign (periumbilical discoloration), Grey Turner's (flank bruising); ↑ amylase/lipase; NPO; pain after fatty foods.
Cirrhosis / Hepatic Encephalopathy	Ascites, jaundice, asterixis, confusion; low-protein diet; lactulose/Rifaximin to reduce ammonia; monitor for bleeding.
Cholecystitis	RUQ pain radiating to right shoulder, nausea after fatty meal, fever, Murphy's sign.
Peptic Ulcer (Gastric vs Duodenal)	Gastric → pain worsens after eating; Duodenal → pain relieved by eating; risk for perforation (board-like abdomen).
Diverticulitis	Avoid seeds/nuts; NPO during acute phase; high-fiber after recovery.
Peritonitis	Rigid abdomen, rebound tenderness, fever, tachycardia; notify provider.
Appendicitis	RLQ pain (McBurney's point); rebound tenderness; do not apply heat.

Gastrointestinal & Hepatic Disorders

DISORDER	KEY NCLEX-FOCUSED POINTS
Dumping Syndrome	Post-gastrectomy; small frequent meals, no fluids with meals, lie down after eating.
Gastrectomy / Post-op	B12 deficiency risk; monitor for dumping syndrome.
IBD (Ulcerative Colitis / Crohn's)	UC → continuous, bloody diarrhea; Crohn's → patchy, malabsorption.
Viral Hepatitis (A–E)	Avoid acetaminophen, rest, proper hygiene; transmission awareness.

B. NUTRITION & DIET SATA

Gastrointestinal & Hepatic Disorders

01	Iron-rich foods – shrimp, meat, spinach (used in infant anemia sets).
02	Low-fat diet – cholecystitis, pancreatitis, hyperlipidemia.
03	Low-protein diet – hepatic encephalopathy (prevent ammonia).
04	High-protein, high-calorie diet – Crohn's, anemia recovery.
05	Ketogenic diet – "when to intervene" (wrong diet listed: high carb/high protein).
06	Gut health foods – kefir/yogurt differentiation (student recall).

C. GASTROINTESTINAL PROCEDURES

Gastrointestinal & Hepatic Disorders

01 Gastrectomy – monitor dumping syndrome, B12 deficiency.

02 Colostomy (Day 1 post-op) – clarify order before irrigating; priority check for leakage.

03 Paracentesis (Ascites) – monitor hypotension, assess for fluid reaccumulation.

04 Endoscopy / Colonoscopy – bowel prep; assess for perforation afterward.

05 NG Tube / PEG Feeding – confirm placement, residual volume, elevate HOB.

D. GI PHARMACOLOGY

Gastrointestinal & Hepatic Disorders

DRUG	KEY FOCUS
Metronidazole	Avoid alcohol (disulfiram reaction).
Rifaximin	For hepatic encephalopathy; reduces ammonia.
Psyllium fiber	Bulk-forming laxative; take with water.
Antacids (Aluminum hydroxide)	Watch for constipation.
Proton Pump Inhibitors / H2 Blockers	Omeprazole, Ranitidine; reduce gastric acid.
Sucralfate	Coats ulcer; give on empty stomach.
Antiemetics	Ondansetron, Metoclopramide (EPS warning).
Pancreatic Enzymes (Pancrelipase)	Take with meals; improves fat digestion.

PICTURE / IMAGE-BASED PROMPTS

Gastrointestinal & Hepatic Disorders

01	Cullen's and Grey Turner's signs (image SATA).
02	Bull's-eye rash (Lyme disease) – GI infection crossover.
03	Pressure ulcer stage 1 vs skin discoloration – infection overlap.

PRIORITIZATION & DELEGATION HOOKS

Gastrointestinal & Hepatic Disorders

01	"Which GI client to see first?" (e.g., rigid abdomen → peritonitis).
02	"Post-cholecystectomy: which finding to report first?" (e.g., jaundice, clay stools).
03	"Post-paracentesis: which client needs reassessment?" (hypotension, dizziness).
04	"Colostomy irrigation order — clarify first."
05	"Hepatic encephalopathy: which diet teaching is incorrect?" (protein intake).

FREQUENTLY REPEATED TOPICS

Gastrointestinal & Hepatic Disorders

"Case: Peritonitis (NGN)."

"Case: Peptic Ulcer (4-case NGN)."

"Case: Cholecystitis (RUQ pain)."

"Rifaximin for hepatic encephalopathy."

"Ketogenic diet – intervention needed if high carb/protein."

"Colostomy day 1 – order clarification."

"Metronidazole – no alcohol."

"Pancrelipase – take with meals."

"Case: Acute Pancreatitis (Cullen & Grey Turner signs)."

"Case: Cirrhosis with ascites."

"Case: Diverticulitis (avoid seeds)."

"Iron-rich foods – SATA."

"Gut health – kefir vs yogurt."

"Dumping syndrome – small, frequent meals."

"Psyllium – take with water."

"Ascites – measure girth and weight."

10

SYSTEM 10

Musculoskeletal & Integumentary Disorders

High Yield

Musculoskeletal & Integumentary Disorders

01	Compartment Syndrome — "3-column question SATA – compartment syndrome, fat embolism, acute osteoarthritis." — "Case study – Compartment Syndrome (33 y/o male in 4-case NGN)." — "Case: Compartment syndrome – pain out of proportion, pallor, pulselessness." Format: NGN / SATA / Multi-scenario case. Focus: 6 Ps (pain, pallor, paresthesia, paralysis, pulselessness, poikilothermia); priority is fasciotomy and neurovascular checks.
02	Total Hip / Knee Arthroplasty — "Below-the-knee amputation post-op management – SATA." — "Total hip arthroplasty – post-op precautions." — "4-client NGN set included 39 y/o hip replacement, 3 days post-op, pain 8/10 – prioritization." Format: NGN / SATA / Prioritization. Focus: Prevent dislocation (no flexion >90°), use abduction pillow, avoid crossing legs, early ambulation, DVT prevention.
03	Fractures & Traction — "Fractures – prioritization." — "Buck's traction – skin traction care." — "Cast care – compartment syndrome watch." Format: Case Study / SATA / Prioritization.
04	Fat Embolism Syndrome (FES) — "3-column SATA – compartment syndrome, fat embolism, acute osteoarthritis." — "Recognize FES triad – dyspnea, confusion, petechial rash." Format: SATA / NGN case pair with trauma.
05	Ankylosing Spondylitis — "Case mention – Ankylosing spondylitis." Format: Stand-alone recognition item (spinal stiffness, posture correction, pain management).
06	Osteoarthritis / Osteoporosis — "3-column question SATA (acute osteoarthritis)." — "Raloxifene – osteoporosis management." Format: SATA / Stand-alone drug item.

Musculoskeletal & Integumentary Disorders

DISORDER	NCLEX-FOCUSED DETAILS
Compartment Syndrome	Assess for 6 Ps; elevate extremity to heart level; loosen cast/dressing; notify HCP.
Fat Embolism Syndrome	Triad: dyspnea, confusion, petechiae on chest/axilla; early post-fracture complication.
Osteoarthritis (OA)	Chronic joint stiffness, pain worse with activity; warm compresses, weight reduction.
Rheumatoid Arthritis (RA)	Autoimmune; morning stiffness; rest during flare.
Osteoporosis	Postmenopausal women; calcium + Vit D; weight-bearing exercise.
Ankylosing Spondylitis	Spinal stiffness, fusion; encourage posture exercises; firm mattress.
Amputation Care	Prevent contractures (lie prone), monitor bleeding, pain phantom sensation.

Musculoskeletal & Integumentary Disorders

01	"Transferring from bed to chair – place chair on stronger side."
02	"Use of crutches – correct gait and safety."
03	"ROM exercises (passive/active)."
04	"Position after hip replacement – avoid flexion, use abduction pillow."
05	"Prevent DVT post-arthroplasty – early ambulation, SCDs."
06	"Arthroplasty: report calf pain (possible DVT)."

C. SKIN INTEGRITY / INTEGUMENTARY

Musculoskeletal & Integumentary Disorders

01	"Lice infestation (pediculosis) – contact precautions."
02	"Bull's-eye rash (Lyme disease) – picture; precautions."
03	"Pressure ulcer staging – picture identification."
04	"Prevention: turn q2h in bed, q15min in chair, avoid donut pillows."
05	"Stage 1 – non-blanchable redness."
06	"Stage 2 – partial-thickness skin loss."
07	"Stage 3 – full-thickness; visible fat."
08	"Stage 4 – exposed bone/tendon."

D. PHARMACOLOGY

Musculoskeletal & Integumentary Disorders

DRUG	KEY NCLEX POINTS
Raloxifene	Osteoporosis; risk for DVT; report calf pain.
Calcitonin / Alendronate	Take on empty stomach, remain upright 30 min.
NSAIDs / Ibuprofen	For OA; monitor GI bleed.
Cyclobenzaprine (Muscle Relaxant)	Drowsiness, avoid driving.
Etanercept / Infliximab	For RA; immunosuppression risk.
Opioids (Morphine)	Monitor RR; naloxone antidote.

PICTURE / IMAGE-BASED PROMPTS

Musculoskeletal & Integumentary Disorders

01	Compartment syndrome limb photo (swollen extremity).
02	Lyme disease bull's-eye rash.
03	Pressure ulcer stage comparison.
04	Use of assistive device (crutch/cane positioning).

PRIORITIZATION & DELEGATION HOOKS

Musculoskeletal & Integumentary Disorders

01 "Which post-fracture client to assess first?" (severe pain unrelieved by meds → compartment syndrome).

02 "Post-arthroplasty client with pain 8/10 – priority?" (assess alignment, DVT signs).

03 "Delegation – assign stable OA client to LPN; UAP can assist with ambulation under supervision."

04 "Who to discharge first?" (stable OA vs fresh hip replacement).

05 "Skin breakdown prevention – who to see first (stage 2 vs reddened area)."

FREQUENTLY REPEATED TOPICS

Musculoskeletal & Integumentary Disorders

"Case: Compartment syndrome – 6 Ps priority."

"Case: Total hip arthroplasty – prevent dislocation."

"Case: Fat embolism – dyspnea, confusion, petechiae."

"Fracture – cast care, traction."

"Buck's traction – maintain alignment."

"Ankylosing spondylitis – posture exercise."

"Osteoarthritis – warm compress, weight reduction."

"Osteoporosis – calcium + Vit D, raloxifene teaching."

"Pressure ulcer prevention – turn q2h."

"Crutch safety – chair on strong side."

"Lice infestation – contact precaution."

"Lyme disease – bull's-eye rash picture."

"DVT prevention – early ambulation."

11

SYSTEM 11

Pediatric Nursing

High Yield & Frequently Repeated

Pediatric Nursing

01	Cystic Fibrosis (CF) — "Case: Cystic fibrosis – airway clearance and nutrition." — "4-case NGN included 5-day-old infant with dehydration and poor feeding." — Focus: Thick mucus, pancreatic enzyme replacement, high-calorie/high-protein diet, chest physiotherapy. Format: NGN / SATA / Prioritization.
02	Kernicterus — "Case: Kernicterus – baby position (picture)." — Focus: Positioning (side-lying), phototherapy safety, bilirubin toxicity prevention. Format: NGN / Image / SATA.
03	Pertussis / Whooping Cough — "Pertussis – antibiotic therapy (Erythromycin)." — "Communicable disease precautions – droplet." Format: Case Study / SATA / Stand-alone.
04	Down Syndrome / Genetic Conditions — "Normal head size, Down syndrome – pedia recall." — "Newborn with brown spots – rule out neurofibromatosis (café-au-lait spots)." Format: NGN / SATA / Picture.
05	Developmental Milestones — "18-month-old prioritization." — "7-year-old developmental milestones." — "2-year-old – appropriate play (parallel play)." — "8-month-old – car seat position." Format: Case / Stand-alone / SATA.
06	Communicable Diseases — "Diphtheria (adult), Shigellosis, Scarlet Fever, Fifth Disease." — "Viral meningitis – cohorting." Format: Case / SATA / Infection control crossover.
07	Pediatric Emergencies — "Dehydration, DHN (infant), fever, poor feeding – included in multi-client NGN." — "Epiglottitis / Croup – airway management, vaccine." — "PDA, RSV, Pertussis – infection control and prioritization." Format: Case Study / Prioritization.

Pediatric Nursing

AGE	KEY MILESTONES
7 years old	Industry vs Inferiority (Erikson); follows rules, peer play.
2 years old	Parallel play, 2–3 word phrases, runs, climbs stairs.
18 months	Walks independently, stacks 3 blocks, imitates chores.
8 months	Rolls both ways, sits unsupported, responds to name.
11 years (female)	Signs of puberty onset.
Play Stages	Solitary (infant), Parallel (toddler), Associative (preschool), Cooperative (school-age).

Pediatric Nursing

DISORDER	NCLEX-FOCUSED NOTES
Cystic Fibrosis	Thick mucus → airway clearance first; pancreatic enzymes with meals; ↑ calories/protein; monitor Na^+ loss (sweat test).
Pertussis (Whooping Cough)	Droplet precaution; antibiotic (Erythromycin/Azithromycin); isolation until 5 days antibiotics completed.
Croup / Epiglottitis	Airway emergency; avoid throat exam; prepare trach set; cool mist O_2.
Kawasaki Disease	IV Immunoglobulin (IVIG) + Aspirin; monitor for cardiac aneurysm.
GBS (Guillain-Barré Syndrome)	SATA – cause post-URI or GI infection; monitor respiratory status.
Meningitis (Bacterial/Viral)	Brudzinski/Kernig signs; droplet precaution; cohort same infection.
Rheumatic Fever	After strep infection; prophylactic Penicillin; cardiac assessment.

Pediatric Nursing

DISORDER	NCLEX-FOCUSED NOTES
Iron Deficiency Anemia	Teach iron-rich foods (shrimp, meats, leafy greens).
Thalassemia Major	Transfusion-dependent; monitor iron overload.
PKU (Phenylketonuria)	Restrict phenylalanine (no aspartame); special formula (Lofenalac).
Autism Spectrum Disorder (ASD)	Repetitive behavior, poor eye contact; maintain routines.
ADHD	Med side effect: decreased appetite, weight loss.
Cerebral Palsy / Spastic Type	Diazepam for spasticity; encourage ROM and safety.

C. PEDIATRIC PROCEDURES / SAFETY

Pediatric Nursing

01	Phototherapy: protect eyes/genitals, turn q2h, monitor temp/hydration.
02	IM injection sites: vastus lateralis for infants.
03	Ear irrigation: pull pinna down & back (<3 yrs), up & back (>3 yrs).
04	Car seat safety: rear-facing until age 2 or weight limit met.
05	Restraint checks: q1h, tie to bed frame, quick-release knot.
06	Post-mortem care: remove tubes only if not for autopsy (appeared in recall).

Pediatric Nursing

DRUG	INDICATION / FOCUS
IVIG + Aspirin	Kawasaki disease.
Diazepam	Spastic cerebral palsy; watch sedation.
Erythromycin	Pertussis or Chlamydia; macrolide class.
Penicillin G	Rheumatic fever, GBS.
Desmopressin (DDAVP)	Diabetes insipidus in pediatric DI.
Iron supplements	Give with juice, avoid milk.

PICTURE / IMAGE-BASED PROMPTS

Pediatric Nursing

01	Kernicterus (side-lying baby).
02	Café-au-lait spots (neurofibromatosis).
03	Brudzinski & Kernig signs (meningitis).
04	Rash patterns – Fifth Disease "slapped face."
05	Car seat positioning diagram.
06	Parallel play photos.

PRIORITIZATION & DELEGATION HOOKS

Pediatric Nursing

01 "Which pediatric client to assess first?" → Airway compromise (Croup/Epiglottitis) > Dehydration infant > Post-vaccine fever.

02 "Who can UAP assist?" → Stable child with cast or CF performing chest PT under RN supervision.

03 "Vaccine contraindication" → Moderate/severe illness; allergy to vaccine component.

04 "Who to cohort?" → Same pathogen (e.g., RSV with RSV).

FREQUENTLY REPEATED TOPICS

Pediatric Nursing

"Case: Cystic Fibrosis – airway and nutrition."

"Case: Pertussis – droplet precaution, antibiotic therapy."

"7-year-old – developmental milestones."

"Croup vaccine – respiratory protection."

"PKU diet – restrict phenylalanine."

"Iron-rich foods SATA."

"GBS – post-URI or GIT infection cause."

"Case: Kernicterus – baby positioning."

"2-year-old – parallel play."

"8-month-old – car seat placement."

"Rheumatic fever – prophylactic Penicillin."

"Kawasaki disease – IVIG + Aspirin."

"Café-au-lait spots – neurofibromatosis."

12

SYSTEM 12

Multisystem, Ethical, and Leadership Integration

High Yield

Multisystem, Ethical, and Leadership Integration

01	Prioritization & Delegation (Top 3 Trend) — "Prioritization – which client to see first." — "Delegation to UAP – SATA: 5 rights of delegation." — "Multi-client case: 4 scenarios – COPD, Anemia Post-fall, DHN infant, Hip Arthroplasty (who to see first)." Format: NGN / SATA / Multi-scenario. Focus: Recognize unstable vs stable, RN-only tasks, delegation safety.
02	Disaster Management / Triage — "Disaster management – who to save first." — "Family disaster plan – ethical obligation of nurse to serve." — "Triage in ER – red/yellow/green/black tag simulation." Format: NGN / Case / Prioritization. Focus: START triage principle — airway, breathing, perfusion, mental status.
03	Incident Reporting / Conflict Management — "Incident report after fall precaution breach." — "Conflict among staff – how to manage." — "Complaints from clients – therapeutic response." Format: Stand-alone / SATA. Focus: Assertive communication, SBAR use, no blame documentation.
04	Ethical Dilemmas — "Question on beneficence and utility." — "Breach of confidentiality – identify action." — "Autonomy and informed consent – emergency exceptions." Format: SATA / Case. Focus: Legal duties, patient rights, ethical reasoning hierarchy.
05	Quality Improvement / EBP — "Question on quality improvement, evidence-based practice, and research application." — "EBP question – how to ensure safe outcomes." Format: Stand-alone / SATA.

A. PRIORITIZATION (WHO TO SEE FIRST)

Multisystem, Ethical, and Leadership Integration

PATTERN	NCLEX-FOCUSED CONCEPT
Stable vs Unstable	Unstable = acute, new, post-op <24 hrs, unexpected findings.
Maslow's Hierarchy	Airway → Breathing → Circulation → Safety → Pain.
ABCs + Safety	Chest pain, difficulty breathing, neuro changes always priority.
First, Next, Immediate	"First" = critical life threat; "Next" = safety/comfort after.
Trend Alert	>70% of scholars reported "Prioritization" as main exam core.

Multisystem, Ethical, and Leadership Integration

TASK TYPE	WHO CAN DO IT
Assessment / Teaching / Evaluation	RN only.
Stable clients, predictable outcomes	LPN can monitor, reinforce teaching.
Basic care (ADLs, vitals, ambulation)	UAP delegation allowed.
5 Rights of Delegation (SATA)	Right Task, Circumstance, Person, Communication, Supervision.
Floating Nurse Assignments	Assign clients within previous expertise area.

Multisystem, Ethical, and Leadership Integration

CONCEPT	EXAMPLE NCLEX FOCUS
Autonomy	Informed consent; right to refuse treatment.
Beneficence	Acting for client's benefit.
Nonmaleficence	Do no harm; avoid negligence.
Justice	Fair and equitable care.
Veracity	Tell the truth.
Utility	Greatest good for greatest number (disaster setting).
Confidentiality	HIPAA; avoid disclosure to unauthorized individuals.

Multisystem, Ethical, and Leadership Integration

01	Informed Consent Exceptions — Emergencies, minors with certain rights (pregnancy, STI).
02	"Incident Report – fall or med error documentation." → Objective, factual, not part of chart, no blame language.
03	"Breach of confidentiality – correct action." → Notify supervisor, protect client privacy.
04	"Malpractice / Negligence scenarios." → Missed assessment, failure to report → RN accountability.
05	"Informed consent in emergency – can be given verbally or implied." → Reported multiple times in October batch.

Multisystem, Ethical, and Leadership Integration

SCENARIO	NCLEX-LINKED TEACHING
Conflict with staff	Use assertive, respectful language; address privately.
Therapeutic Communication	Avoid false reassurance; use open-ended questions.
Crisis & Complaint Management	Focus on client safety, listen actively, report through chain of command.
Transcultural Nursing	Respect religious & cultural beliefs (e.g., Muslim care refusal).
Caregiver Exhaustion	Refer to support group (not medication).
Reinforce vs Remind	"Reinforce" = RN follow-up teaching confirmation.

Multisystem, Ethical, and Leadership Integration

CONCEPT	KEY APPLICATION
Evidence-Based Practice (EBP)	Use best evidence + clinical expertise + patient preference.
Sentinel Event Reporting	Immediate action, root-cause analysis.
Quality Improvement Process	Plan → Do → Study → Act (PDSA cycle).
Safety Goals	Hand hygiene, correct ID, read-back verbal orders.

PICTURE / EXHIBIT PROMPTS

Multisystem, Ethical, and Leadership Integration

01	Incident report forms (sample item).
02	EHR excerpt: identify HIPAA violation.
03	Delegation chart (who to assign).
04	Triage color coding visual.
05	Maslow pyramid graphic question.

PRIORITIZATION & DELEGATION HOOK EXAMPLES

Multisystem, Ethical, and Leadership Integration

01	"Which client to assess first?" (Chest pain vs Pain 3/10 vs Anxious → Airway first).
02	"Which task can be delegated to UAP?" (Vital signs, bathing, ambulation of stable client).
03	"Floating nurse from pedia to MS – which assignment?" (Pediatric case only).
04	"Incident: fall with minor abrasion – next nursing action?" (Assess, document, complete report).
05	"Which statement shows breach of confidentiality?" (Sharing info with another client).

FREQUENTLY REPEATED TOPICS

Multisystem, Ethical, and Leadership Integration

"Prioritization – who to see first."

"Delegation – 5 rights (Task, Circumstance, Person, Communication, Supervision)."

"Case: 4-client NGN (COPD, Anemia, DHN, Hip Arthroplasty)."

"Disaster triage – who to save first."

"Ethics – beneficence, utility."

"Conflict with staff – assertive communication."

"Incident report – objective documentation."

"Breach of confidentiality – report to supervisor."

"Informed consent – exception in emergency."

"Quality improvement – evidence-based practice application."

A Letter to My Dear Future USRNs, AURNs, and CARNs

My dearest students,

If you're reading this, it means you've already come this far — and that alone is something to be proud of. The journey to becoming an NCLEX-RN passer isn't easy, but remember: the hardest climb always leads to the most beautiful view.

Every lecture you attended, every Qbank you finished, every time you chose to study instead of rest — they all brought you closer to the dream you once whispered in prayer. And that dream? It's no longer distant. It's right here, waiting for you to claim it.

When you sit for your exam, remember what I always say:

Breathe. Think. Trust your training.

You have spent months learning, reflecting, and mastering not just content — but clinical judgment, critical thinking, and heart.

You are ready.

This file you hold isn't just a list of topics — it's a testament to every student who believed, fought, and finally saw their name followed by those two powerful letters: R.N. (USA, CANADA, AUSTRALIA)

So as you open your laptop or walk into the testing center, remind yourself:

"Kaya today."

Because you've already done the work.

Because you already are the nurse you've dreamed to be.

Because you deserve to pass — not by luck, but by preparation and purpose.

Now go and claim it.

Claim your title. Claim your story. Claim your victory.

I'll be right here, cheering you on — every step of the way. 💙

With pride and faith in you,

Kristina Marie Parulan-Andaya, BSN, PHRN, USRN, MAN-NA — International Nursing Educator • "Kaya today."



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